

Journey Strategy

Goal

Reduce patient friction, reduce operational breakdowns, and lower the amount of coordination work patients must do themselves.

What The Data Suggests

- Social burdens cluster together rather than appear alone.
- The strongest hardship bundle is **Financial Resource Strain + Food insecurity + Transportation Needs + Utilities + Housing Stability**.
- **Depression** and **stress** are strongly linked.
- **Major depressive disorder, recurrent** has the clearest relationship with multiple hardship domains, especially **stress, intimate partner violence, and Food insecurity**.
- These patterns suggest the biggest journey failures happen at handoffs, follow-up, and cross-team coordination points.

Core Strategy

Shift from a visit-centered model to a journey-centered model.

That means:

- identify high-friction encounters early
- assign the next step to the system, not the patient
- use domain signals to personalize support
- intervene hardest at transitions between visits

Priority Journey Problems

1. Patients are doing too much coordination themselves

Examples:

- calling other offices on their own
- arranging transportation on their own
- managing referrals across teams
- figuring out follow-up steps from fragmented instructions

2. High-burden patients face multiple barriers at once

A patient with transportation, food, and housing strain will not benefit much from solving only one barrier.

3. Behavioral-health-related journeys need integrated support

For depression-related encounters, clinical treatment alone is not enough. The journey likely needs social and operational support built in.

Recommended Actions

1. Create a high-friction journey segment

Flag encounters with:

- high-risk domain bundles
- recurrent depression or other high-burden diagnoses
- repeated follow-up needs

Use these flags to route patients into enhanced support workflows.

2. Remove self-coordination at handoffs

- schedule follow-up before the patient leaves
- make referrals directly instead of asking patients to initiate them

- give one clear next-step plan
- assign ownership for the next action to staff or workflow, not the patient

3. Add domain-based support triggers

- **Transportation Needs**: bundle appointments, offer virtual options, improve logistics support
- **Food insecurity** or **Financial Resource Strain**: route to resource support and simplify billing-related friction
- **Housing Stability**: escalate navigation help and reduce dependence on complicated self-managed follow-up
- **Depression** and **stress**: use proactive outreach and lower-burden communication

4. Pilot a navigator-supported pathway

Start with one cohort:

- recurrent depression plus high social burden

Then test:

- proactive outreach
- coordinated follow-up
- social-resource connection
- simplified care-plan communication

Metrics To Track

- follow-up completion rate
- no-show rate
- referral completion rate
- time to next completed encounter
- repeat acute utilization
- percent of high-friction encounters with documented next-step ownership

Bottom Line

The best opportunity is not a generic customer-experience program. It is a targeted journey redesign for patients whose clinical needs and social burdens overlap.

If the system takes over more of the coordination work, especially at transitions, patients will experience less friction and operations will run more reliably.